

Progress Notes

Signed Apr 14, 2026

Progress Notes by Shraddha M Dalwadi, MD at 4/14/2026 8:37 PM



DEPARTMENT OF RADIATION ONCOLOGY FOLLOW-UP VISIT

NAME: Karen Clayborne
DOB: 1/11/1966
MRN: 60937438

IDENTIFICATION SENTENCE:

KAREN CLAYBORNE is a 60 y.o. year old female presenting for radiation therapy with palliative intent for diagnosis of secondary malignant neoplasm of brain (C79.31). She is here today for post RT follow up after the below treatment:

Delivered Treatment

The following treatment has been delivered to date:

Course: 1

- Volume: Left Frontal
- Technique: SRT
- Energy: 6XFFF
- Fractions: 3 / 3
- Total Dose: 2700 cGy / 2700 cGy
- Frequency: SRT - VMAT, Stereoscopic x-ray - Daily

Treatment Dates and Response

Treatment start date: 02/04/2026

Treatment end date: 02/06/2026

INTERVAL HISTORY:

Overall doing well but having some pressure headaches and mild dizziness

No visual/hearing changes, seizures

Family members expressed concern about systemic plan as patient is seeing multiple specialists

Pt not interested in radiation at this time
Pt would like to go to Houston to get a second opinion
No fever, chills, new LAD

PAST MEDICAL HISTORY:

Past Medical History

Past Medical History:

Diagnosis	Date
• EGFR-related lung cancer (HCC)	10/2024
• Headache, chronic migraine without aura	
• Long term (current) use of anticoagulants	
• Lung disease	
• Malignant neoplasm of brain (HCC)	03/2025

PAST SURGICAL HISTORY:

Past Surgical History

Past Surgical History:

Procedure	Laterality	Date
• HX MYOMECTOMY		2005
• PLEURAL TAP- ORDER TRAY & STERILE GLOVES		10/2024

MEDICATIONS:

Medications Ordered Prior to Encounter

Current Outpatient Medications on File Prior to Encounter

Medication	Sig	Dispense	Refill
• amlodipine (NORVASC) 5 MG Oral tablet	Take 1 tablet (5 mg total) by mouth in the morning.		
• amoxicillin-clavulanate (AUGMENTIN) 875-125 MG Oral per tablet	in the morning and in the evening.		
• carvedilol (COREG) 6.25 MG Oral tablet	Take 1 tablet (6.25 mg total) by mouth in the morning and 1 tablet (6.25 mg total) before bedtime.		
• glipiZIDE (GLUCOTROL) 5 MG Oral tablet	TAKE 1/2 TABLET BY MOUTH TWICE DAILY WITH MEAL		
• losartan (COZAAR) 50	Take 1 tablet (50		

MG Oral tablet	mg total) by mouth in the morning.
• Lazertinib Mesylate (LAZCLUZE) 240 MG Oral TABS	Oral; Duration: 30 Days
• metformin (GLUCOPHAGE) 1000 MG Oral tablet	Take 1 tablet (1,000 mg total) by mouth in the morning and 1 tablet (1,000 mg total) in the evening.
• NIFEdipine (PROCARDIA XL) 30 MG Oral tablet	Take 1 tablet (30 mg total) by mouth in the morning and 1 tablet (30 mg total) before bedtime.
• ELIQUIS 2.5 MG Oral tablet	Take 1 tablet (2.5 mg total) by mouth in the morning and 1 tablet (2.5 mg total) before bedtime.
• TAGRISSO 80 MG Oral TABS	take 1 tablet by mouth daily with or without food
• dexAMETHasone (DECADRON) 1 MG Oral tablet	Take 1 tablet (1 mg total) by mouth in the morning. (Patient not taking: Reported on 4/14/2026)
• dexAMETHasone (DECADRON) 4 MG Oral tablet	Take 1 tablet (4 mg total) by mouth in the morning and 1 tablet (4 mg total) before bedtime. (Patient not taking: Reported on 4/14/2026)

No current facility-administered medications on file prior to encounter.

ALLERGIES:

No Known Allergies

SOCIAL HISTORY:

Social History

Social History

Tobacco Use

- Smoking status: Never
- Passive exposure: Never
- Smokeless tobacco: Never

Substance Use Topics

- Alcohol use: Never
- Drug use: Never

FAMILY HISTORY OF CANCER:

Family History

Family History

Problem	Relation	Name	Age of Onset
• Esophageal Cancer	Father		
• Lung Cancer	Paternal Grandfather		

REVIEW OF SYSTEMS:

Reviewed with pertinent findings as per HPI, otherwise 14 point review of systems was negative.

PHYSICAL EXAM:

VITALS: Visit Vitals

BP	(!) 167/117
Pulse	86
Ht	5' 3" (1.6 m)
Wt	218 lb (98.9 kg)
SpO2	91%
BMI	38.62 kg/m ²
Smoking Status	Never
BSA	2.1 m ²

Wt Readings from Last 3 Encounters:

04/14/26	218 lb (98.9 kg)
01/28/26	214 lb (97.1 kg)
01/14/26	214 lb (97.1 kg)

BP Readings from Last 3 Encounters:

04/14/26 (!) 167/117

01/28/26 156/89

01/14/26 156/89

GENERAL: KPS 90. Well appearing female in NAD.

HEENT: Normocephalic, atraumatic. No scleral icterus.

LYMPHATICS: No cervical, axillary, or supraclavicular adenopathy palpable bilaterally.

CARDIOVASCULAR: No peripheral edema or cyanosis.

PULMONARY: No acute respiratory distress.

BACK: No gross deformity.

ABDOMEN: Grossly nondistended.

NEUROLOGIC: Cranial nerves II-XII grossly intact. Strength 5/5 in upper and lower extremities bilaterally. Gait normal without assistive devices.

MR BRAIN WOW CONTRAST 70553

Order: 76972494

Impression

1. Mild heterogeneous nodular lesion involving the left paramedian lower vermis/medial cerebellar hemisphere has increased in size with increase in the surrounding edema.
2. Punctate areas of nodular enhancement in the left lateral and lower cerebellum and in the bilateral occipital lobes are mildly more pronounced from the prior exam. These findings are concerning for disease worsening.
3. The previously seen heterogeneously enhancing lesion in the left paramedian frontal lobe has decreased in size but decrease in surrounding edema and interval gradient susceptibilities which may suggest posttreatment changes.
4. The heterogeneous enhancing lesion involving the upper and mid cerebellar vermis, closely related to the posterior and lateral margins of the fourth ventricle, is mildly increased in size but the surrounding edema does not show prominent increase and interval gradient susceptibility is are seen. Findings may suggest treatment-related changes and needs clinical correlation.
5. Ventricles are grossly stable in size. No interval intracranial hemorrhage, prominent vasogenic edema, or acute infarction is otherwise seen in the rest of the brain.

I, Manoj Kumar, have personally interpreted the image(s) and dictated this examination or I have reviewed the image(s) as well as the residents interpretation and agree with the interpretation.

Narrative

UNIVERSITY HEALTH

4502 MEDICAL DRIVE, SAN ANTONIO, TX 78229

PH: 210-358-8532

PROCEDURE(S): MR BRAIN W AND WO IV CONTRAST

ACCESSION(S): UH5919153

DATE OF EXAM: 4/10/2026 3:38 PM

REFERRING PHYSICIAN: SHRADDHA M DALWADI

EXAM: MRI brain with and without IV contrast.

TECHNIQUE: Multiplanar, multisequence MRI of the brain was performed both before and after the administration of intravenous gadolinium contrast.

CONTRAST: 10 mL Gadavist

HISTORY: Malignant neoplasm of unspecified part of unspecified bronchus or lung (CMS/HCC) [C34.90]

Malignant neoplasm of unspecified part of unspecified bronchus or lung (CMS/HCC) [C34.90]

COMPARISONS: MR brain perfusion with and without contrast 1/24/2026

FINDINGS:

Brain parenchyma/contrast enhancement: No acute infarction or acute intraparenchymal hemorrhage. Periventricular and deep subcortical white matter T2/FLAIR hyperintensities consistent with mild chronic microvascular ischemic changes. No new definite enhancing lesion is identified.

Heterogenously enhancing lesions as follows:

- * Midline cerebellar vermis lesion is increased in size now measuring 2.2 x 2.6 x 2.2 cm (previously measuring 1.4 x 2.4 x 1.3 cm). Mild increase in degree of adjacent perilesional edema. Interval increase in amount of intralesional susceptibility artifact.
- * Left lower cerebellar lesion is increased in size now measuring 1.1 x 1.0 x 1.1 cm (previously measuring up to 0.6 cm). Mild perilesional edema.
- * Left frontal lobe lesion is significantly decreased in size now measuring up to 9 mm (previously measuring up to 1.5 cm). Significant interval decrease in amount of perilesional edema. This lesion demonstrates susceptibility artifact.
- * 2-3 mm enhancing nodular lesions in the left lateral cerebellum and in the bilateral occipital lobes are mildly more pronounced from the prior exam when they were very subtle and small.

Contrast Enhancement: No abnormal enhancement to suggest tumor or infection.

Ventricular system: No hydrocephalus or midline shift.

Extra axial spaces: No epidural, subdural, or subarachnoid hemorrhage. No basal cistern effacement.

Vascular system: No loss of the major vascular flow voids.

Visualized paranasal sinuses: Mild bilateral maxillary sinus mucosal thickening. The bilateral mastoid air cell complexes are clear.

Visualized orbits: No ocular or retrobulbar pathology.

Calvarium and skull base: No osseous destruction.

Pituitary and Pineal Glands: No mass lesion.

Soft tissues: No significant abnormality.

Images on Order 76972494

Image Retrieve

The full-size image has not yet been retrieved from an outside organization. To retrieve, click the link below.

[Scan on 4/10/2026: MR brain with and without IV contrast](#)

LABS:

CBC w/Diff		
Lab Results		
Component	Value	Date
WHITEBLOOD	9.24	01/14/2026
REDBLOOD	4.67	01/14/2026
HEMOGLOBIN	13.5	01/14/2026
HEMATOCRIT	42.1	01/14/2026
MCV	90.1	01/14/2026
MCH	28.9	01/14/2026
MCHC	32.1	01/14/2026
RDW	15.5 (H)	01/14/2026
PLATELETCNT	209	01/14/2026
MPV	11.6	01/14/2026
ANC	83.8 (H)	01/14/2026
LYMPHS	7.6 (L)	01/14/2026
MONOCYTE	7.7	01/14/2026

EOSINOPHILS	0.3 (L)	01/14/2026
ABSOLUTELYM	0.70 (L)	01/14/2026
ABSOMONOCYT	0.71	01/14/2026
ABSOEOSINOP	0.03 (L)	01/14/2026
ABSOBASOPHI	<0.03	01/14/2026

Comprehensive Metabolic Panel	
GLUCOSE (mg/dL)	PROTEIN, TOTAL (g/dL)
Date Value	Date Value
01/14/2026 122	01/14/202 7.8
	6
UREA NITROGEN (mg/dL)	ALBUMIN (g/dL)
Date Value	Date Value
01/14/2026 15.0	01/14/202 3.2 (L)
	6
CREATININE (mg/dL)	GLOBULIN (g/dL)
Date Value	Date Value
01/14/2026 0.96	01/14/202 4.6
	6
EGFR (mL/min/1.73 m²)	BILIRUBIN, TOTAL (mg/dL)
Date Value	Date Value
01/14/2026 68	01/14/202 0.3
	6
SODIUM (mmol/L)	ALKALINE PHOSPHATASE (U/L)
Date Value	Date Value
01/14/2026 141	01/14/202 100
	6
POTASSIUM (mmol/L)	ASPARTATE AMINOTRANSFERASE (U/L)
Date Value	Date Value
01/14/2026 4.1	01/14/202 17
	6
CHLORIDE (mmol/L)	ALANINE AMINOTRANSFERASE (U/L)
Date Value	Date Value
01/14/2026 107	01/14/202 15
CARBON DIOXIDE (mmol/L)	
Date Value	
01/14/2026 26	

PT/INR

No results found for: "CEA"

ASSESSMENT/PLAN:

Overall stable since completion of radiation but recent imaging 4/10 shows mixed response overall.

- MMP referral: pt needs a PCP
- Psyc support offered
- Neurosurgery referral urgently to eval options
- Encouraged pt to discuss with Dr. Kelly - however pt is seeking a second opinion at MDA and has an outside medonc and may not continue care at UT
- Would prefer not to radiate again without TB discussion
- ER precautions given

Future Appointments

Date	Time	Provider	Department	Center
4/16/2026	10:30 AM	LAB DRAWS	CTRC WC	CTRC
4/16/2026	11:30 AM	Kelly, William, MD	MCCURS4THF LR	CTRC

I personally spent a total of 30 minutes on the date of this encounter: (1) reviewing the patient's health record (including outside records, clinical notes, laboratory results, pathology reports, and diagnostic imaging); (2) performing a history and physical examination; (3) providing counseling and education; (4) documenting clinical information in the health record; (5) writing orders; and (6) coordinating care with others involved in the patient's care.

Thank you for the consult.

Shraddha Mahesh Dalwadi, MD, MBA

Assistant Professor

Department of Radiation Oncology

UTHSCSA MD Anderson Mays Cancer Center

